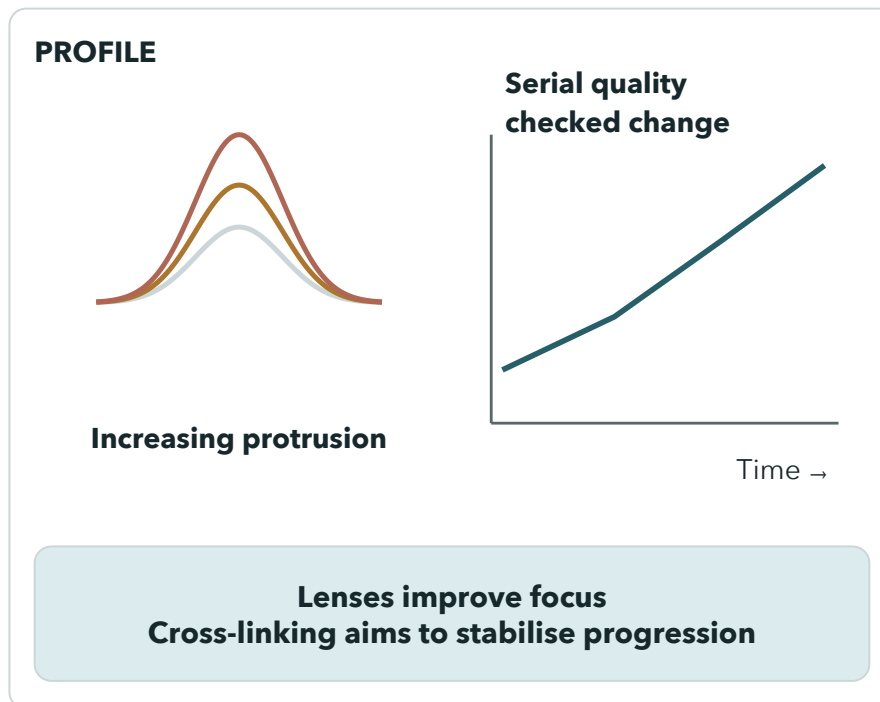


Corneal instability: shape versus surface

Stabilising ectasia and preventing recurrent erosion solve different problems.

Progression is more than a new prescription

A | Optics and structural stability



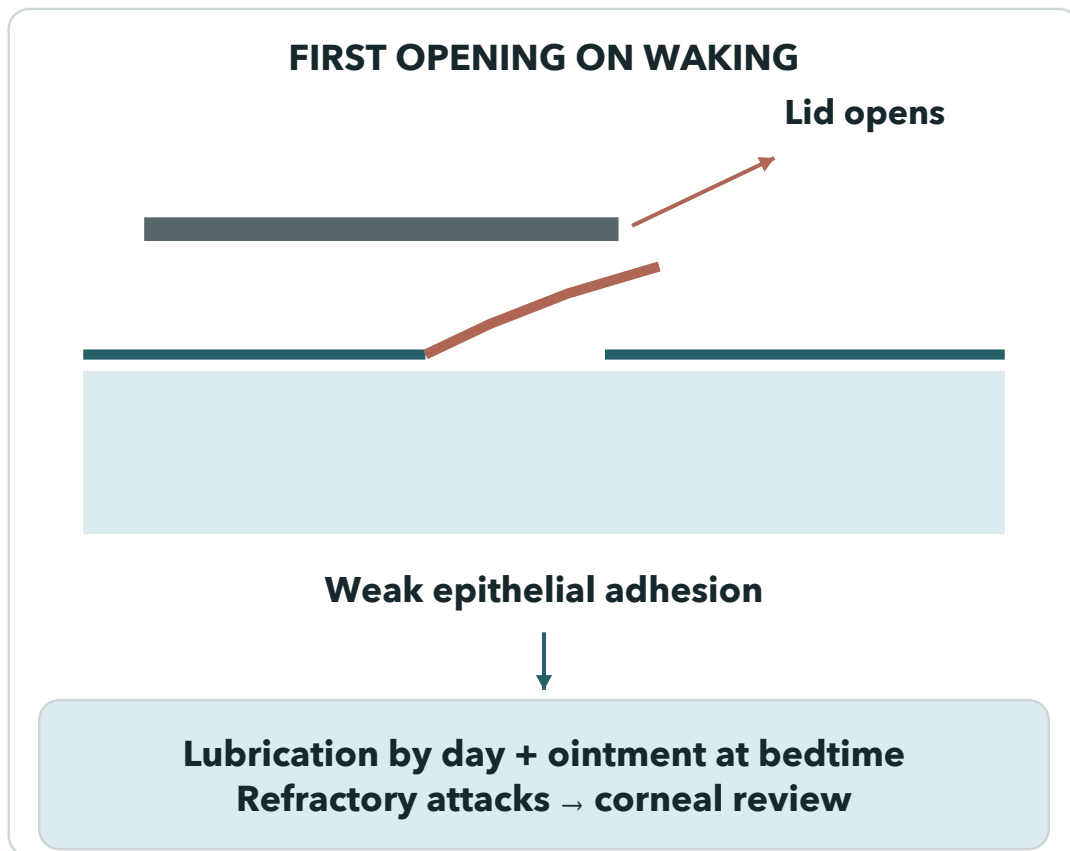
Original corneal profile and serial-measurement schematic. Curves illustrate a trend, not diagnostic numerical thresholds.

Follow the right outcome

Question	Evidence or action
Can the patient see better?	Refraction, spectacles or rigid lenses assess optical rehabilitation.
Is the cornea changing?	Compare quality-controlled serial tomography, steepening and thinning after lens-wear effects are considered.
Documented progression?	Refer for cross-linking suitability; thickness, scarring and protocol safety matter.
Can cross-linking replace lenses?	Its main aim is stability; useful focusing correction may still be needed.

Recurrent erosion: prevent morning shearing

B | Why the first blink hurts



Original lid-surface mechanism diagram. Weak epithelial adhesion can allow an adherent lid to shear the epithelium on waking.

Between attacks

Once the acute defect heals, daytime lubrication and bedtime lubricating or hypertonic ointment are reasonable first conservative measures. Address dry eye and lid problems. Repeated attacks despite an adequate regimen need corneal review for selected procedures or supervised bandage-lens treatment. A new infiltrate, progressive defect or unexpected visual loss requires re-examination rather than assuming every painful episode is the familiar erosion. Do not use repeated topical anaesthetic to suppress attacks. A young patient may retain useful contact-lens acuity while tomography shows genuine ectatic progression. Stability and acuity are separate outcomes. A healed recurrent erosion is another example: the acute antibiotic course does not replace a prevention plan for the next morning.